



Division of Provider Services
& Quality Assurance
P.O. Box 8059, Slot S404
Little Rock, AR 72203-8059

July 31, 2025

Dean Hill, Administrator
Delta Family Health And Fitness Center For Childre
815 E St Louis
Hamburg, AR 71646

Dear Mr. Hill:

On July 18, 2025 a Complaint Investigation survey was conducted at your facility by the Office of Long Term Care to determine if your facility was in compliance with Federal requirements for Psychiatric Residential Treatment Facilities participating in the Medicaid (Title XIX) Program. This survey found that your facility had deficiencies requiring correction/substantial correction prior to a revisit as specified in the attached CMS-2567.

Plan of Correction

A POC must be submitted within 10 calendar days of you receipt of the Statement of Deficiencies. Failure to submit a POC may result in termination. Include a completion date for each deficiency cited.

Stephanie Lester, Reviewer
OLTC, Survey & Certification Section
PO Box 8059, Slot S404
Little Rock, AR 72201-4608
(501) 396-6013
email to stephanie.lester@dhs.arkansas.gov.

Your Plan of Correction must also include the following:

- a. Address how the corrective action will be accomplished for those residents found to have been affected by the deficient practice;
- b. Address how the facility will identify other residents having the potential to be affected by the same deficient practice;
- c. Address what measures will be put into place or systemic changes will be made to ensure that the deficient practice will not recur;
- d. Indicate how the facility plans to monitor its performance to make sure that solutions are sustained. The facility must develop plan for ensuring that correction is achieved and sustained. This plan must be implemented, and the corrective action evaluated for its effectiveness.

e. Include dates when corrective action will be completed. The corrective action completion dates must be acceptable to the State. Your facility is ultimately accountable for its own compliance. The plan of correction will serve as the facility's allegation of compliance. Unless otherwise stated on the PoC, the last completion date will be the date of alleged compliance.

Informal Dispute Resolution

In accordance with 42 CFR § 488.331, you have one opportunity to question deficiencies through an informal dispute resolution (IDR) process. To obtain an IDR, you must send your written request to Health Facility Services, Arkansas Department of Health within ten (10) calendar days from receipt of the Statement of Deficiencies. The request must state the specific deficiencies the facility wishes to challenge. The request should also state whether the facility wants the IDR to be performed by a telephone conference call, record review, or a face-to-face meeting.

If the facility will be represented by an Attorney, please notify ADH in your IDR request and provide the name and contact number for the Attorney. Additionally, please notify DHS, at OLTC2@dhs.arkansas.gov, of your intent to be represented by an Attorney. Failure to do so may result in a delay of your hearing.

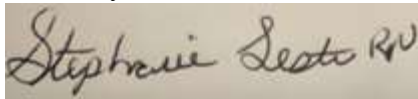
An incomplete informal dispute resolution procedure will not delay the effective date of any enforcement action or the requirement for timely submission of an acceptable plan of correction. Informal dispute resolution in no way is to be construed as a formal evidentiary hearing. It is an informal administrative process to discuss the findings.

Please submit your request to:

**IDR/IIDR Program Coordinator
Health Facilities Services
5800 West 10th Street, Suite 400
Little Rock, AR 72204
Phone: 501-661-2201
ADH.HFS@Arkansas.gov**

If you have any questions, please contact your Reviewer.

Sincerely,



for Jeff Rosenbaum

Stephanie Lester, RN QA Reviewer
DPSQA/Office of Long Term Care
Survey & Certification Section

sl

cc: DRA

Approved
08/06/2025
SL

PRINTED: 07/31/2025
FORM APPROVED
OMB NO. 0938-0391

DEPARTMENT OF HEALTH AND HUMAN SERVICES
CENTERS FOR MEDICARE & MEDICAID SERVICES

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 04L112	(X2) MULTIPLE CONSTRUCTION A. BUILDING _____ B. WING _____		(X3) DATE SURVEY COMPLETED C 07/18/2025
NAME OF PROVIDER OR SUPPLIER DELTA FAMILY HEALTH AND FITNESS CENTER FOR CHILDRE			STREET ADDRESS, CITY, STATE, ZIP CODE 815 E ST LOUIS HAMBURG, AR 71646		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETION DATE	
N 000	Initial Comments Note: The CMS-2567 (Statement of Deficiencies) is an official, legal document. All information must remain unchanged except for entering the plan of correction, correction dates, and the signature space. Any discrepancy in the original deficiency citation(s) will be reported to the Dallas Regional Office (RO) for referral to the Office of the Inspector General (OIG) for possible fraud. If information is inadvertently changed by the provider/supplier, the State Survey Agency (SA) should be notified immediately. Complaint # AR00039471 was not in compliance with deficiencies cited at N128 and N189. Complaint # AR00039478 was not in compliance with deficiencies cited at N128 and N189. The facility was not in compliance with §483, Subpart G - Conditions of Participation for Psychiatric Residential Treatment Center. N 128 PROTECTION OF RESIDENTS CFR(s): 483.356(a)(3) Restraint or seclusion must not result in harm or injury to the resident and must be used only- This ELEMENT is not met as evidenced by: Based on record review, interview and facility policy review the facility failed to prevent injuries during a restraint for two (Client #1 and Client #2) of four clients reviewed for safety during restraints. The findings include: Client #1 A review of Client #1's Master Treatment Plan	N 000			
		N 128	On 07/14/2025, the CPI Instructor began providing training/refreshers to all staff outlining proper CPI techniques to prevent harm or injury to a client. The training includes aspects of CPI that are crucial in preventing client injury such as: team approach, use of appropriate interventions to ensure welfare, safety and security of clients, circumstances that indicate the need for physical hold as a last resort. The CPI Instructor will review camera footage of physical holds biweekly and document findings. All inappropriate initiation or use of physical holds will be reported to the Program Director/Clinical Director who will be responsible for:		

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

Don Will

Administrator

8/6/25

Any deficiency statement ending with an asterisk (*) denotes a deficiency which the institution may be excused from correcting providing it is determined that other safeguards provide sufficient protection to the patients. (See instructions.) Except for nursing homes, the findings stated above are disclosable 90 days following the date of survey whether or not a plan of correction is provided. For nursing homes, the above findings and plans of correction are disclosable 14 days following the date these documents are made available to the facility. If deficiencies are cited, an approved plan of correction is requisite to continued program participation.

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N 128	Continued From page 1 updated on 07/08/2025, revealed Client #1 had diagnoses which included intense irritability, frequent temper outbursts, major depressive disorder, disruptive impulse-control and conduct disorder. A review of a Restraint Order Form (ROF) dated 04/02/2025, reflected that Client #1 was placed in a personal hold restraint at 5:35 PM and released at 5:45 PM. A review of the Face-to-Face Assessment for Injury located within the ROF packet dated 04/02/2025 revealed the nurse observed that Client #1 was crying and asked for medication for mouth pain. The nurse offered Client #1 over-the-counter pain medication. Client #1 refused the medication. Documentation on the Face-to-Face Assessment indicated Client #1 had braces on their teeth and that they refused to let the nurse look inside their mouth to see what caused the discomfort. A review of an Internal Investigation Log dated 06/06/2025, revealed that on 06/06/2025 Client #1 reported an allegation of an incident that occurred on 04/02/2025. Client #1 reported to their caseworker that they had received a busted lip, which had also bled, when Client #1 had been placed in a restraint by Behavior Coach (BC) #1 A review of the ROF packet dated 04/02/2025 of the nurse's assessment noted no injury was observed to Client #1's face. On 07/17/2025 at 2:56 PM, during an interview, Client #1 stated that while in their room they were told by BC #1 to stop throwing a foam mat in the air. Client #1 stated that they deliberately defied	N 128	placing staff in question on suspension until completion of in house investigation, notification of reporting entities if warranted, enforcing appropriate disciplinary action. Documentation of findings during video review will be reported on a quarterly basis at DFC Quality Assurance Team Meetings until a threshold of 100% compliance has been achieved for a consecutive 3 month period.	8/8/2025	

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N 128	Continued From page 2 BC #1 and continued to throw the mat. Client #1 reported BC #1 could not do a standing hold restraint on Client #1 because they were throwing objects, so they went down on the floor in a restraint and Client #1's lip was pinched in their braces and busted. Client #1 asked if the investigation of the incident could be dropped because staff were mad about the investigation. Client #1 stated that they refused to let the nurse look at their lip after the restraint. On 07/17/2025 at 4:08 PM, during an interview, Behavior Coach Supervisor (BCS) #2 confirmed that Client #1 had been placed in a Crisis Prevention Institute (CPI) prone position restraint. BCS #2 stated they did not see the restraint take place. BSC #2 reported she was standing in the doorway calling for help. BCS #2 then turned and went into the room after BC #1 had Client #1 restrained on the floor. BCS #2 stated Client #1 had complained of their mouth hurting during the restraint and " spat multiple times on the floor and it appeared to be a mixture of spit and blood, " however, Client #1 refused to let the nurse evaluate their mouth when they were released from the restraint. On 07/17/2025 at 6:31 PM, during a phone interview, BC #1 confirmed that he initiated the restraint for Client #1 on 04/02/2025, before BCS #2 came to assist. BC #1 stated he was trying to do a standing restraint, but the client pulled away and dropped down to the floor and BC #1 followed Client #1 to the floor. BC #1 reported that the client was in the prone position and denied any knowledge of Client #1 having received injuries. Client #2	N 128			

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N 128	Continued From page 3 A review of Client # 2's Master Treatment Plan, revealed Client #2 had diagnoses which included intense irritability, frequent temper outbursts, other specified trauma and stress related disorder, child sexual abuse, child neglect, and upbringing away from parent. A review of a ROF dated 06/07/25, revealed Client #2 was placed in a physical restraint at 3:02 PM, and released at 3:22 PM, due to physical aggression and trying to elope. A chemical restraint was administered at 3:10 PM, due to agitation. A review of a Face-to-Face Assessment for injury located within the ROF packet completed at 4:40 PM on 06/07/25, noted that Client #2 complained of pain to their left elbow. Client #2 also had a left upper arm abrasion, scratches to left and right facial area, scratches to left middle finger, injection site times two to hips, and bruises to Client #2's nose. A review of a Nursing Progress Note dated 06/07/25 at 5:20 PM, stated that Client #2 identified another area on their arm that was injured during the restraint. Per the Nursing Progress Note " this injury was not identified at the time of assessment at 4:40 PM and was 'believed' to be self-inflicted. " The Nursing Progress Note also indicated the client had what appeared to be a bruise on the nose. On 07/17/25 at 2:50 PM, during an interview, Client #2 reported they were hurt during a restraint by BC #1. Client #2 reported they were put into two separate restraints on the same day and BC #1 shoved Client #2's face in the ground and hurt Client #2's nose. Client #2 also indicated	N 128			

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N 128	Continued From page 4 that during the second restraint, Client #2's elbow " popped " which caused pain. Client #2 indicated that during the second restraint they were also scratched, and pressure was put on their knee while lying face down. On 07/17/2025 at 6:31 PM, during a phone interview, BC #1 reported he was not involved in the restraint on 06/07/2025, when Client #2 received bruising to the nose. He also stated there was a second restraint on 06/07/2025 in which the client was rubbing their nose on a foam pad that was on the floor. On 07/17/2025 at 5:00 PM, during an interview, the Administrator confirmed there should always be two staff members during the transition of a client from a standing restraint to a prone position restraint. A review of a facility policy titled, " Procedures for the use of Personal or Chemical Restraints, " with last revision date of 01/2024, revealed in part that personal restraints should not cause harm or pain to the resident.	N 128			
N 189	POST INTERVENTION DEBRIEFINGS CFR(s): 483.370(b) Within 24 hours after the use of restraint or seclusion, all staff involved in the emergency safety intervention, and appropriate supervisory and administrative staff, must conduct a debriefing session that includes, at a minimum, a review and discussion of - 483.370(b)(1) The emergency safety situation that required the intervention, including discussion of the precipitating factors that led up	N 189	On 7/10/2025 the Director of Nursing initiated training of all nursing staff on completion of client and staff post restraint debriefings per CFR 483.370(b). The Director of Nursing will be responsible for review of all restraint documents to ensure client and staff debriefings are documented within 24 hours of restraint. The Director of Nursing will present results of debriefing reviews to DFC Quality Assurance Team quarterly		

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N 189	Continued From page 5 to the intervention; This ELEMENT is not met as evidenced by: Based on record review, interview and facility policy review the facility failed to ensure a staff debriefing was conducted within 24 hours after an emergency safety intervention was implemented for one (Client #3) of four clients reviewed for receiving emergency safety interventions. The findings include: Review of Client #3's Master Treatment Plan updated on 06/03/2025, revealed the client had diagnoses which included intense irritability and frequent temper outbursts, disruptive impulse-control, conduct disorder, and attention deficit hyperactivity disorder. Review of a Restraint Order Form (ROF) dated 04/30/2025 PM, reflected that Client #3 was placed in a personal hold restraint at 9:00 AM, and released at 9:12 AM. Review of a Staff and Client Debriefing Form located within the ROF packet dated 04/30/2025, revealed no documentation to indicate that staff had conducted a debriefing. All areas under the section labeled "Staff Debriefing" were blank. Review of a Nurse's Note located within the ROF dated 04/30/2025, reflected Client #3 had been placed in a personal restraint and given a chemical restraint. The Nurse's Note did not indicate a debriefing had been conducted on 04/30/2025, and there was not another Nurse's Note until 05/04/2025.	N 189	until a threshold of 100% compliance is achieved for 6 consecutive months.	8/8/2025	

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N 189	Continued From page 6 On 07/18/2025 at 9:56 AM, the Assistant Administrator provided a Critical Incident Report (CIR) Form which indicated Client #3 had been placed into a personal hold from 9:00 AM, until 9:35 AM. The CIR did not include three staff members who were named on the ROF dated 04/30/2025 at 9:00 AM, as being involved in the restraint. The CIR did not have a date or time to indicate when it was completed, nor did it reflect that a debriefing had been conducted with the staff members named in the above-mentioned ROF. The Assistant Administrator confirmed the staff members involved in the restraint were not listed on the CIR. On 07/18/2025 at 9:41 AM, during an interview the Assistant Administrator confirmed the ROF, the Nurses' Notes, and the CIR would be the only internal documents that would reflect client and staff debriefings after a client was placed in a restraint. Review of a facility policy titled, Procedures for the use of Personal or Chemical Restraints with a revision date of 01/2024, specified in part that all staff involved in an emergency safety intervention must participate in a debriefing session to discuss what led up to the restraint, what could have been done differently to prevent the restraint, what less-restrictive techniques could be used in the future and evaluation of the events to ensure that all of the client 's rights were maintained during the restraint. The policy specified that a staff debriefing must be conducted within 24 hours after the use of an emergency safety intervention.	N 189			



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& Quality Assurance
P.O. Box 8059, Slot S404
Little Rock, AR 72203-8059

August 6, 2025

Dean Hill, Administrator
Delta Family Health And Fitness Center For Childre
815 E St Louis
Hamburg, AR 71646

Dear Mr. Hill:

On July 18, 2025 we conducted a complaint survey at your facility. You have alleged that the deficiencies cited on that survey have been corrected. We are accepting your allegation of compliance and have approved your plan of correction and presume that you will achieve substantial correction by August 8, 2025.

If you have any questions, please contact your reviewer: **Stephanie Lester at 501-396-6013 or email to: stephanie.lester@dhs.arkansas.gov.**

Sincerely,

A handwritten signature in cursive script that reads "Stephanie Lester RN". The signature is written in dark ink on a light-colored, slightly textured background.

for Jeff Rosenbaum

Stephanie Lester, RN QA Reviewer
DPSQA/Office of Long Term Care
Survey & Certification Section

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